2026
NCBAHM™ Candidate Preparation Handbook
for
Acupuncture Certification

All NCBAHM certification programs are accredited by the NCCA®

NCBAHM™
1199 N. Fairfax Street, Suite 220
Alexandria, VA 22314
(202) 381-1140 (Phone) • (202) 381-1141 (Fax)



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NCBAHM™ Mission, Vision, and Core Values

NCBAHM™ Mission

To assure the safety and well-being of the public and to advance and advocate for the professional practice of NCBAHM Board-Certified Acupuncturists™ by promoting established national standards focused on competence and credentialing.

NCBAHM™ Vision

NCBAHM Board-Certified Acupuncturists™ will be globally recognized and integral to person-centered healthcare and accessible to all members of the public. NCBAHM Board Certification will be nationally recognized by all employers and government entities as the standard for acupuncturists.

NCBAHM™ Core Values

Through its commitment to lifelong learning and the highest quality of credentialing standards for public safety, the NCBAHM upholds the values of integrity, community, service, inclusiveness, advocacy, and accountability in all of its interactions and relationships.

The NCBAHM programs in Acupuncture & Herbal Medicine, Acupuncture, and Chinese Herbology are accredited by the National Commission for Certifying Agencies (NCCA) and carry the NCCA seal.

Non-Discrimination Policy

The NCBAHM does not discriminate on the basis of race, color, age, gender, sexual orientation, political or religious beliefs, handicap, marital status, national origin, or ancestry.

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About NCBAHM™

Founded in 1982 as a non-profit certification organization, the National Certification Board for Acupuncture and Herbal Medicine (NCBAHM™), formally known as National Certification Commission for Acupuncture and Oriental Medicine (NCCAOM), is widely accepted as the most influential leader in the field of certification for acupuncture and herbal medicine. There are currently over 21,000 active NCBAHM Diplomates (holders of NCBAHM certification). The NCBAHM is responsible for the development and administration of the Acupuncture, Chinese Herbology, and Acupuncture & Herbal Medicine Certification Programs. The NCBAHM evaluates and attests to the competency of its National Board-Certified Acupuncturists™ through rigorous eligibility standards and demonstration and assessment of the core knowledge, skills and abilities expected for an entry level practitioner of acupuncture and herbal medicine.

To learn more about the NCBAHM visit the About tab on the NCBAHM website.

NCBAHM Certification Programs

The NCBAHM certification programs include Acupuncture & Herbal Medicine (AHM), Acupuncture (Ac), and Chinese Herbology (CH). The NCBAHM Asian Bodywork Therapy (ABT) program no longer offers examinations, however, the current NCBAHM ABT Diplomates may keep their NCBAHM Board Certification by maintaining their continued competencies. The three certification programs are accredited by the National Commission for Certifying Agencies (NCCA), whose standards are consistent with *the Standards for Educational and Psychological Testing (AERA, APA, & NCME, 1999)* and are applicable to all professions and industries.

Acupuncture & Herbal Medicine
Dipl. A.H.M. (NCBAHM)™

Acupuncture
Dipl. Ac. (NCBAHM)™

Chinese Herbology
Dipl. C.H. (NCBAHM)™

Asian Bodywork Therapy
Dipl. ABT (NCBAHM)™

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NCBAHM™ Code of Ethics

All practitioners certified by the NCBAHM are committed to responsible and ethical practice, to the growth of the profession within the broad spectrum of American healthcare, and to their own professional growth. All Diplomates, applicants and candidates for certification are bound by the *NCBAHM™ Code of Ethics (PDF)* and *NCBAHM™ Grounds for Professional Discipline (PDF)*.

Benefits of NCBAHM™ Certification

All the benefits of Certification can be found at *Diplomate Benefits* on the NCBAHM website.

Introduction

Passing the Foundations of Oriental Medicine, Biomedicine, and Acupuncture with Point Location examinations are required to becoming an NCBAHM™ Diplomate of Acupuncture, and demonstrates that a candidate possesses the core knowledge, skills, and abilities expected of an entry-level practitioner of Acupuncture. This Handbook is designed to assist candidates preparing to take these examinations. All examination preparation materials published by the NCBAHM, and available information related to the examinations are contained within this document.

Please note that candidates for NCBAHM™ Certification in Acupuncture are applicants who have met all academic and/or training requirements and are qualified by one of the established eligibility routes published in the *NCBAHM™ Certification Handbook (NCBAHM Website)*. If the candidate's application has not been approved and/or there are questions about the application process, please refer to the *NCBAHM™ Certification Handbook* or email info@ncbahm.org for more information. This document focuses on assisting candidates in preparing for and taking the examinations as well as understanding examination processes from content development through score reporting.

The NCBAHM encourages candidates to read this Handbook in its entirety to familiarize themselves with all examination policies and procedures.

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Examination Development

Standards

The Acupuncture, Chinese Herbology, and Acupuncture & Herbal Medicine certification programs are accredited by the National Commission for Certifying Agencies (NCCA) (website), an organization that enforces industry standards for professional certification programs to help ensure the health, welfare, and safety of the public. For the

Acupuncture certification program to remain accredited by the NCCA, the NCBAHM must adhere to strict national standards outlined in the *Standards for the Accreditation of Certification Programs*. All NCBAHM examinations must meet test development Standards set forth by the NCCA.

Job Analysis

The foundation of a fair, relevant, and legally defensible professional certification program is based on a well-designed and executed job analysis (JA) study. A JA study establishes the link between competencies assessed by the examination and what practitioners actually perform on-the-job. This link ensures that all examination pass/fail decisions correlate to practitioner competence. To keep examination content relevant to current practice, the NCBAHM conducts a JA study at a minimum of every five (5) to seven (7) years.

The current examination content outlines contained in this Handbook are based on the results from the 2024 Job Analysis study (NCBAHM Website). A JA Panel of subject matter experts (SMEs), consisting of NCBAHM Diplomates who represent the diversity in practice (e.g., gender, ethnicity, geographic region of the United States, work settings, and years of experience) are recruited. The JA Panel creates a list of tasks and competency statements that describe the knowledge, skills, and abilities required for safe and competent practice as an entry-level acupuncturist. This list is converted into a survey that is distributed to NCBAHM Diplomates and licensed acupuncturists to solicit feedback on the relative importance of each task and frequency of use in practice. Additionally, survey respondents are invited to submit additional tasks that did not appear on the survey.

The JA survey results are analyzed by a psychometrician to determine the criticality of each task and ensure representativeness of the survey responses. Results of the statistical analyses are then presented to the JA Panel of SMEs, who establish and apply decision criteria to determine which tasks should be tested on the examination and appear on the new content outlines. Through this rigorous process, the JA study provides validity

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evidence for the content of the NCBAHM examinations and establishes the link between examination content and what practitioners perform on-the-job.

This Handbook provides the content outlines for each examination required for NCBAHM™ Certification in Acupuncture. Each content outline describes the detailed competency/task statements and the distribution of test questions (items) across the various content areas of the examination, also known as domains. Task statements provide the candidate with additional information about the knowledge tested within each domain.

Item Writing

After a new content outline is established based on the results of a JA study, examination items are written for each task statement. Each item must be "linked" to a domain and task statement listed on a content outline. Items cannot appear on an examination if they do not test knowledge covered in the content outline.

The NCBAHM™ Examination Content Development Committees (ECDCs) creates new items for the certification examinations. The ECDCs are composed of subject matter experts (SMEs) representing the diversity of practice in terms of gender, ethnicity, geographic region of the United States, work settings, and years of experience. The ECDCs convene for the purposes of writing, reviewing, and revising examination items to meet strict content guidelines and test construction standards. New SMEs are trained to write high-quality, multiple-choice items by experienced SMEs and the NCBAHM Testing staff. During item writing training, new SMEs learn about appropriate item formats and types, and factors that influence the cognitive complexity of an item. The NCBAHM provides SMEs with item writing guidelines, which includes instructions for writing high-quality items and what types of items to avoid (e.g., negatively worded questions that use "not" or "except"). Sample items representative of ones seen on the examinations appear later in this Handbook.

Item Review

The NCBAHM™ Examination Review Committee (ERC) reviews all accepted items from the Examination Content Development Committees (ECDCs) and approves them for use on an examination. Items must link to a task statement on the content outline and be current and accurate. Items must have a single correct answer that can be verified by at least one reference. Revisions are made to an item until all SMEs agree that the item meets NCBAHM guidelines and is appropriate for use on the examination.

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All examinations include scored items as well as a limited number of pretest items. In other words, pretest items are utilized to determine the validity of using a question as a scored item on future examinations. Pretest items do not count toward a candidate's total score. All new items must meet psychometric standards before being used in a scored position on an examination. Items not meeting psychometric standards are flagged, and additional review must be conducted by the SMEs. Flagged items that have been revised are then again pretested before they can be included in scored positions on the examination so as not to disadvantage candidates.

The NCBAHM applies strict standards, and every effort is made to avoid errors in examination items and ensure that all items used in scoring are fair and appropriate.

Examination Administration

Examination Day – Be Prepared!

Plan to arrive at the test center early. Candidates who arrive after their appointed time will not be allowed to take the examination, and all fees paid for that examination will be forfeited. Candidates are advised to carefully review Pearson VUE's policies before arriving at the test center and be sure to follow all instructions provided by test center staff during the examination and the check-in/out processes.

Candidates must bring the required Identification to the test center:

1. NCBAHM Authorization to Test (ATT) Letter
2. Current driver's license OR government ID with photo and signature
3. A second, current personal ID (school, work, credit card, etc.) with a signature

The name on the pictured ID and the ATT Letter must exactly match, and the ID must be current/valid (unexpired). If not, the candidate will not be allowed to take the examination, and all fees paid for that examination will be forfeited. Pearson VUE has the right to refuse IDs that are damaged (e.g., cracked, broken) or otherwise deemed unacceptable according to their policies. If a candidate has misplaced their NCBAHM Authorization to Test (ATT) Letter, they may reprint the letter from their online account located on the NCBAHM Certification Portal.

The Pearson VUE staff will review the candidate's identification, take the candidate's photograph, and conduct a palm vein scan. The palm vein scan is a standard procedure that ensures:

1. each test taker has a single record,

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2. representation of a virtually error-free identification system that is non-intrusive to the user,
3. increased accuracy and security at the check-in and check-out procedures, and
4. verification of each candidate as they enter and exit the test center.

All personal items (e.g., wallet/purse, phones, coat/jacket, food), including but not limited to wearable technology (e.g., watches, smart glasses, etc.) are prohibited in the testing room. If smart glasses are prescription, candidates must bring a standard pair of glasses for use during testing as they will not be able to use smart glasses. If a candidate did not bring standard prescription glasses, they will be denied entry, the appointment will be marked as a "no-show", and the candidate will be required to obtain a new authorization to test with NCBAHM. Candidates will be asked to empty their pockets and store any personal items in a test center locker before being admitted to the testing room. The Test Center will provide candidates with a ring binder containing five erasable sheets and a marker that will be collected at the end of the examination. Please note that for security purposes, there is video surveillance throughout the test center and candidates will be audio and videotaped during the check-in and check-out processes and during their examination. For additional information, please visit the NCBAHM Certification Process and the Pearson VUE websites.

Inclement Weather, Power Failure or Emergency

NCBAHM and Pearson VUE will determine whether circumstances warrant the cancellation of an examination administration at a particular test center site due to inclement weather, unforeseen emergencies or "Acts of God". Every attempt is made to administer the examination as scheduled; however, should an examination be canceled at a testing center, all scheduled candidates will receive notification by email or phone regarding rescheduling in a timely manner. Additional examination fees will not be incurred if the examination is canceled by NCBAHM or Pearson VUE.

On rare occasions, technical difficulties at the test center may be encountered. Candidates should immediately notify the test center proctor if any computer irregularities arise. If the test center experiences an unexpected, temporary power outage during examination administration, test center back-up systems are in place to ensure testing data is preserved. The candidate's examination will resume from the point where it was interrupted, and the candidate continues with the examination. Every effort is made to administer the examination as scheduled. However, should an examination be canceled at a testing center, all scheduled candidates will be notified by e-mail or phone with rescheduling instructions.

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Confidentiality – IMPORTANT!

After a candidate completes the check-in process and is seated at a computer, the candidate will be asked to review a checklist of rules and must agree to the *NCBAHM™ Non-Disclosure Agreement (NDA) and General Terms of Use for NCBAHM Examinations* presented on the computer screen. Candidates have five (5) minutes to review the NDA and select *"I agree"* to begin the examination. **If a candidate does not accept the NDA within the allotted 5 minutes, the examination will terminate and the candidate will be locked out of the computer. The candidate will not be allowed to continue with their examination, and their examination fees will not be refunded.**

According to Pearson VUE rules, **candidates may not begin writing on the supplied note board until the examination has started.** Candidates should not use their note boards until after they have agreed to the Non-Disclosure Agreement (NDA). **If the examination terminates because the candidate was writing on their note board during the NDA, Pearson VUE is unable to start the examination again, the candidate will forfeit all the fees paid for the examination, and the candidate will have to reregister and pay full fees for the examination if they wish to take it again.** See below for the complete terms of the NDA which will be presented to candidates at the test center.

Non-Disclosure Agreement (NDA) and General Terms of Use for NCBAHM Examinations

"I have read and understand the Examination Instructions. I have agreed to abide by the NCBAHM™ Code of Ethics and Grounds for Professional Discipline and acknowledge that if I am caught cheating on this examination, including the sharing of information after the examination is complete; I will be subject to review by the Professional Ethics and Disciplinary Committee of NCBAHM. If I am found to have violated the Code of Ethics and Grounds for Professional Discipline, I understand that my scores will be canceled, and I may not have the opportunity to test again.

Additionally, I understand that this examination is confidential and is protected by trade secret law. It is made available solely for the purpose of becoming certified by NCBAHM. I am expressly prohibited from disclosing, publishing, reproducing, or transmitting this examination, in whole or in part, in any form or by any means, verbal or written, electronic or mechanical, for any purpose.

I am the candidate whose name appears on the initial screen and as an affirmation to the Statement of Acknowledgement I signed when submitting my application. I acknowledge that I am prohibited from transmitting information about NCBAHM examination questions or

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content in any form to any person or entity. I also acknowledge that if I suspect a violation on the part of others, it is my responsibility to report these actions to the NCBAHM."

The NCBAHM is committed to the integrity and security of its examination process. Candidates have a duty to maintain strict confidentiality with respect to the content of the examinations and comply with all examination security policies and procedures. Any breach of confidentiality that may compromise the security of the examination content (e.g., sharing or receiving information about the examination from another person including teachers/professors, before, during, or after the examination) will be grounds for disciplinary action, including but not limited to denial or revocation of certification by the NCBAHM. Likewise, any act, either intentional or unintentional, that violates examination rules will be grounds for disciplinary action by the NCBAHM. Proctors are responsible for reporting to the NCBAHM any candidates who:

1. may have caused a disruption or interruption during the examination period,
2. violated procedural rules before or during the examination, and/or
3. appeared to engage in a method of cheating or seeking unfair advantage either before or during the examination.

The NCBAHM will review these reports and apply appropriate disciplinary sanctions. Strict adherence to NCBAHM examination policies and procedures is enforced without exception.

Participation in any examination policy violation(s) occurring during, or in connection with, an examination may be sufficient cause for the NCBAHM, at its sole discretion, to terminate the candidate's participation, invalidate examination results, seek monetary compensation, deny the application for certification, or take other appropriate action at any time, including but not limited to disciplinary action such as suspension or revocation of certification.

Violations of examination policies include but are not limited to the following actions:

1. Talking with anyone, except the proctors, inside or outside the examination room during the examination period.
2. Giving information about the examination to another person before, during, or after the examination. This includes discussion with professors/teachers and memorizing sections of the examination for use by others.
3. Phone calls or communication by electronic or other means with anyone or anything inside or outside of the examination room during the examination period.

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4. Passing or attempting to pass information of any type to another candidate during or after the examination administration.
5. Possessing any extraneous items during the examination period, including but not limited to books, paper (including scrap paper), notes, note cards, sticky notes, measuring devices (including rulers and calculators), dictionaries (electronic or printed), beepers, cell phones, cameras, and other electronic devices.
6. Communicating with anyone other than a proctor in the case of a site irregularity in which the candidate must exit the examination room (e.g., fire drill, power outage, medical emergency, etc.), except communications as required because of the emergency. This includes communicating with other candidates while waiting to re-enter the test site, looking at notes or books, and/or re-entering the test site without the expressed permission of the proctors.
7. Accessing notes, cell phones, calculators, beepers, other electronic devices, or individuals from any location either inside or outside the examination room including bathrooms, cars, snack areas, etc.
8. Writing on the desk, other furniture, clothing, or any body part in the examination room before or during the examination.
9. Allowing visitors into unauthorized areas of the examination site.
10. Impersonating, or attempting to impersonate another candidate, or allowing another person to take the examination on behalf of the candidate.
11. Failure to report any examination irregularities, cheating, or other inappropriate behavior.

Cheating of any kind that has taken place before, during or after the examination must be reported. It is the candidate's responsibility to report to NCBAHM any site irregularities or testing violations before, during, or after the examination takes place.

Examination Format

The Foundations of Oriental Medicine, Biomedicine, and Acupuncture with Point Location examinations are administered in English in computer adaptive format, commonly referred to as computerized adaptive testing (CAT). Each examination consists of 100 multiple-choice questions with a 2.5-hour time limit.

In an adaptive examination, questions are selected for candidates according to the assigned domain percentages from the examination content outline (NCBAHM Website). Within each domain, questions are selected to match the candidate's ability. What this means is that a candidate's correct or incorrect response to a question determines the next question

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they receive. If a question is answered correctly, the next question presented is more difficult. If a question is answered incorrectly, the next question presented is easier. Because the correct or incorrect response to each question is used to select the next question, candidates are unable to change their responses to previous questions.

Examination Scoring and Results

Examination Scoring

There is no predetermined number of items that must be answered correctly to pass an adaptive examination. It is not the number of correct answers that determines whether a candidate passes, but their overall score based on the difficulty of the items answered correctly. The overall score required to pass the examination is set by a group of practicing, licensed, and Board-certified acupuncturists during a process called standard setting. For additional information, read *Standard Setting, Scaling and Equating document (PDF)* accessible on the "Examination Results" section from the Examination Process page on the NCBAHM website.

A candidate's raw score (the total number of items answered correctly), taking into consideration item difficulty, is transformed into a scaled score. Because each candidate answers a unique set of questions on an adaptive examination, scaled scores are reported to provide a direct comparison of performance across candidates and examinations. This allows candidates to be held to the same passing standard regardless of which questions they receive. Scaled scores are measures of candidate performance, and the higher the score, the better the performance. Scaled scores range from 1 to 99 with 70 designated as the passing score. Scaled scores do not represent the percentage of questions answered correctly on the examination.

Examination Results Notification

Upon completion of an adaptive format examination at a Pearson VUE test center, candidates receive **preliminary** notification on-screen regarding whether they were successful or unsuccessful in passing the examination. These results are **preliminary** and are verified by a third-party testing company before becoming **official**. The official result letter is uploaded to the candidate's online account on the NCBAHM Certification Portal within 7-10 days after the examination date and is available for download. Candidates can track their examination results by accessing their online account located on the NCBAHM Certification Portal.

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The official pass result letter does not display a numerical score. Candidates who are unsuccessful in passing an examination receive a result letter that exhibits the overall scaled score and performance breakdown across the major content areas/domains of the examination. This information is provided with the sole purpose of helping them understand areas of relative strength and weakness in preparation for their next examination attempt.

The candidate's examination results are released to their schools and the Accreditation Commission for Acupuncture and Herbal Medicine (ACAHM). Score information is used in aggregate to evaluate a program's performance, which is required as part of the ACAHM accreditation process. Candidates can request that their examination results be sent to their state licensing Board directly from their online account located on the NCBAHM Certification Portal.

Examination Score Verification

A candidate can request to have their examination score verified within 30 days of receiving preliminary notification that they were unsuccessful in passing an examination. The score verification process is a manual check of the computer's scoring, conducted by NCBAHM Testing staff, to ensure that all responses were accurately recorded. **Score verification does not include a review of examination content or reconsideration of the correct answer to any item. Individual items and examination content will not be discussed or considered during the score verification process.** Candidates requesting a score verification must complete and submit the NCBAHM™ Score Verification Request Form (NCBAHM website). An invoice will be issued in the candidate's online account located on the NCBAHM Certification Portal where payment will be applied. There is a nominal fee per examination for score verification.

Please note that the NCBAHM does not encourage score verification requests. The NCBAHM enforces strict quality control procedures to ensure examination results are accurate before they are released to candidates. Every examination is scored independently by two professional testing companies to ensure each item is scored accurately and the overall score is correct. In addition, both testing companies have multiple checks in place to flag anomalies in test data that require investigation. Due to the thoroughness of the NCBAHM's examination scoring procedures, no errors have ever been identified from a score verification request.

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Examination Content Review

An unsuccessful examination result alone is not sufficient grounds to submit a review. At the test center, candidates can file an incident report regarding examination content concerns with the test center proctor while the item is still fresh in their mind. The test center proctor is only allowed to record the question (item) number as they are not allowed to record any examination content specifics or description of the examination items. Be sure to obtain a copy of the incident report number. A candidate may submit questions or comments about specific aspects of the examination content, in writing, to the NCBAHM (examcontent@ncbahm.org) within 30 days of taking the examination. Only individual items will be reviewed, and reviews are not conducted on a candidate's entire examination. The written inquiry must include the candidate's name, NCBAHM ID, email address, test date and location, examination taken, and a description of the specific item or concern (e.g., an item involving a pregnant patient could have more than one indicated correct answer or no correct answer is indicated).

The request will first be reviewed by the NCBAHM Testing Department to determine if the submitted item(s) or comment(s) is to be presented to the NCBAHM™ Examination Review Committee for a final determination. Please note that submission of a request does not automatically lead to a rescoring of the examination or Committee review. If the item(s) under review is/are found to have a discrepancy, the candidate's examination will be rescored. The candidate will be notified in writing within 45 business days of any resulting action of the inquiry. All determinations of the NCBAHM™ Examination Review Committee are final.

Please note that no information regarding the specific item(s) will be discussed with candidates, including whether a candidate's response was correct or incorrect. The ***NCBAHM never releases copies of examinations or individual examination items.*** This is considered best practice within the licensure/certification testing industry and the NCBAHM must protect the integrity of its examination – most especially its content. As the NCBAHM's mission is to ensure the safety and wellbeing of the public, we take the security of our examination content very seriously and therefore never share any specific information about questions with candidates.

It is imperative that the candidate refrain from discussing the content of the examination with anyone (including teachers, instructors) other than the NCABHM Testing Department. Doing so is a violation of the *NCBAHM™ Non-Disclosure Agreement and the NCBAHM™ Code of Ethics*. The NCBAHM is committed to the integrity and security

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of its examinations. Candidates have a duty to maintain strict confidentiality with respect to the content of the examinations and comply with all examination security policies and procedures. Any breach of confidentiality that may compromise the security of the examination content (e.g., sharing or receiving information about the examination from another person including teachers/professors, before, during, or after the examination) will be grounds for disciplinary action, including but not limited to denial or revocation of certification by the NCBAHM.

Retaking an Examination

The NCBAHM strongly encourages any candidate who was unsuccessful in passing an examination to seek guidance on how to prepare for their next attempt. Speak with school program directors, a faculty member, or mentor. The NCBAHM Examination Content Outlines and other resources in this Handbook are the best materials to use to prepare for the examination. Visit the NCBAHM Exam Preparation Center (NCBAHM Website) to access the Content Outlines, Official NCBAHM Practice Tests™ (NCBAHM Website), and other examination preparation tools. The NCBAHM Testing staff cannot provide any additional guidance on how to prepare for the examination, nor can they recommend any specific courses for educational remediation.

45 Day Wait Rule

A candidate who is unsuccessful in passing an NCBAHM examination upon 1 - 4 attempts must wait 45 days after their last attempt before retaking the same examination module. Candidates will not be allowed to reschedule their examination before the 45-day waiting period ends for any reason. Wait times for additional attempts vary. Additional details can be found at Re-Taking an Examination (NCBAHM Website).

Examination Attempts

Candidates have five (5) opportunities to successfully pass an examination. After the fifth unsuccessful attempt to pass an NCBAHM examination (inclusive of all formats), the candidate must meet additional educational requirements based on multiple factors (i.e., certification application validity, etc.) to receive additional examination attempts. Please refer to NCBAHM's Retake policy on the website to identify requirements candidates must satisfy to continue testing. Please email examattempts@ncbahm.org for further information.

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2026 Content Outlines

Based on the job analysis conducted in 2024, the content outlines for the Certification in Acupuncture are included below. Starting September 1, 2026, all examinations given on or after that date will use these content outlines until the next job analysis study is complete. The Acupuncture Certification will follow the content outlines for the examinations listed: Foundations of Oriental Medicine, Biomedicine, and Acupuncture with Point Location.

The Foundations of Oriental Medicine Content Outline

Effective September 1, 2026

Note to Candidate: This document serves as a guide to assist in examination preparation for candidates who have met NCBAHM™ eligibility requirements. Below is the content outline for the Foundations of Oriental Medicine examination.

Domain I. Patient Assessment Methods (28% of Exam)

A. Observation / Looking *(Wang)* (6%)
   1. Identify the manifestation of Spirit (Shen) (e.g., complexion, general behavior).
   2. Assess the appearance of the eyes, nose, ears, mouth, lips, throat, face (e.g., color, moisture, texture, corresponding organ locations).
   3. Identify manifestations of the tongue (e.g., body, coating, sublingual veins).
   4. Assess the appearance of the body (e.g., shape, movement, head, hair, neck, back, chest, abdomen, extremities, blood vessels, skin, nails).
   5. Assess the appearance of excretions (e.g., mucus, saliva, sweat, discharge, stool, urine, blood).

B. Auscultation and Olfaction / Listening and Smelling *(Wen)* (3%)
   1. Assess respiratory sounds, voice, tonal qualities, and speech.
   2. Identify odors of the body, breath, excretions, secretions (e.g., urine, stool, vaginal discharge, sweat, wound exudates).

C. Inquiry / Asking *(Wen)* (10%)
   1. Identify presenting complaints (e.g., onset, location, aggravation).

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2. Formulate additional questions based on patient's responses and examination findings.
3. Conduct a Review of Systems (ROS) utilizing "Ten Questions" *(Shi Wen)* (e.g., pain, energy level, sweating, body sensations) and other approaches.
4. Identify etiological factors, including internal (e.g., emotions), external (e.g., environment) and miscellaneous (e.g., diet, occupation) factors.
5. Assess personal and family health history (e.g., past diagnoses, concurrent treatments).

D. Palpation / Touching *(Qie)* (9%)

1. Differentiate characteristics of the radial pulses (e.g., rate, depth, strength).
2. Identify findings from palpation of other body areas (e.g., abdomen, torso, limbs) and channels (e.g., temperature, texture, pain, tissue quality, edema).

Domain II. Pathomechanisms and Pattern Differentiation (38% of Exam)

1. Analyze assessment findings to establish pattern differentiation and underlying pathomechanism(s) based on the following theories:
   a. *Yin/Yang*
   b. *Five Elements (Wu Xing)*
   c. *Eight Principles* (*Ba Gang*) (e.g., interior, exterior, heat, cold)
   d. *Qi, Blood, Body Fluids (Qi, Xue, Jin Ye)*
   e. *Channel (Jing Luo)* (e.g., 12 primary channels, eight extraordinary channels, Luo-connecting channels)
   f. *Organ (Zang Fu)*
   g. *Six Stages (e.g., Tai Yang, Yang Ming, Jue Yin)*
   h. *Four Levels (Wei, Qi, Ying, Xue)*
   i. *Triple Burner (San Jiao)*
   j. *Internal etiology* (e.g., seven emotions, stress)
   k. *External etiology* (e.g., wind, damp, cold, dryness, heat, summer heat, pestilential Qi (Li Qi))
   l. *Miscellaneous etiology* (e.g., diet, lifestyle, trauma)
   m. *Chinese organ clock*

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Domain III. Treatment Planning (34% of Exam)

1. Formulate appropriate treatment principles based upon the following theories:
   a. *Yin/Yang*
   b. *Five Elements* (*Wu Xing*)
   c. *Eight Principles (Ba Gang)* (e.g., interior/exterior, heat/cold)
   d. *Qi, Blood, Body Fluids (Qi, Xue, Jin Ye)*
   e. *Channel (Jing Luo)* (e.g., 12 primary channels, extraordinary channels, Luo-connecting channels)
   f. *Organ (Zang Fu)*
   g. *Six Stages* (e.g., *Tai Yang*, *Yang Ming*, *Jue Yin*)
   h. *Four Levels (Wei, Qi, Ying, Xue)*
   i. *Triple Burner (San Jiao)*
   j. *Internal etiology* (e.g., seven emotions, stress)
   k. *External etiology* (e.g., wind, damp, cold, dryness, heat, summer heat, pestilential Qi (Li Qi))
   l. *Miscellaneous etiology* (e.g., diet, lifestyle, trauma)
2. Establish treatment priorities (e.g., Root and Branch *[Ben Biao],* acute vs. chronic) and develop a treatment plan.
3. Modify the treatment plan based on patient's response, disease progression, and lifestyle factors.
4. Explain assessment findings and the treatment plan to the patient.
5. Counsel the patient on self-care home management techniques to prevent disease and maintain health (e.g., *Qi Gong*, *Tai Ji,* breathing techniques).
6. Provide dietary recommendations according to Chinese medicine (e.g., temperature, taste, functions).

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The Biomedicine Content Outline

Effective September 1, 2026

Note to Candidate: This document serves as a guide to assist in examination preparation for candidates who have met NCBAHM™ eligibility requirements. Below is the content outline for the Biomedicine examination.

Please note: In regard to Clean Needle Technique (CNT), the Biomedicine module focuses on universal precautions and emergency situations in comparison to the Acupuncture with Point Location module which focuses on actual needling and its emergencies (e.g., needle angle and depth).

Domain I. Biomedical Sciences (18% of Exam)

A. Anatomy, physiology, and pathophysiology (6%)
   1. Identify anatomical structures and functions of body systems from a biomedical perspective.
   2. Differentiate between normal and abnormal structures and functions of body systems from a biomedical perspective.
   3. Recognize signs, symptoms, and co-morbidities associated with common medical conditions.[1]

B. Pharmacology (7%)
   1. Identify common pharmaceutical categories and their functions.[2]
   2. Recognize interactions, side effects and adverse effects of pharmaceuticals.

C. Supplements (5%)
   1. Identify commonly used supplements and their functions.[3]
   2. Recognize interactions, side effects and adverse effects of supplements.
   3. Identify clinically significant supplement – pharmaceutical interactions.
   4. Recognize signs and symptoms associated with abnormal levels of commonly used supplements.

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[1] See Appendix A: Medical Conditions.
[2] See Appendix B: Pharmaceuticals.
[3] See Appendix C: Supplements.

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Domain II. Patient Evaluation (26% of Exam)

A. Patient history (4%)
   1. Conduct a whole-person health interview (e.g., personal health history, social history, Activities of Daily Living (ADLs), functional assessment, mental health screening).
   2. Organize information obtained during interview into appropriate sections of the patient history.

B. Physical examination (14%)
   1. Recognize different types of physical examinations (e.g., vital signs, orthopedic, neurological).
   2. Identify appropriate application of physical examination methods (e.g., observation, auscultation, palpation).
   3. Identify orthopedic examination techniques (e.g., range of motion, muscle strength testing, special tests, including orthopedic tests).
   4. Identify neurological examination techniques (e.g., assessment of cognitive function, evaluation of cranial nerves, sensory and motor function, dermatomal testing, deep tendon reflexes)
   5. Recognize how different types of physical examinations are performed.
   6. Interpret clinically significant findings from different types of examinations.

C. Diagnostic tests (8%)
   1. Identify indications for common medical imaging (e.g., x-ray, MRI, CT, PET, colonoscopy, cystoscopy, bronchoscopy, ultrasound).
   2. Recognize the clinical significance of findings from medical imaging.
   3. Identify indications for common laboratory tests (e.g., complete blood count, basic metabolic panel, thyroid panel).
   4. Recognize the clinical significance of findings from laboratory tests.
   5. Identify indications for other diagnostic tests (e.g., EMG, EKG, biopsy).
   6. Recognize the clinical significance of findings from other diagnostic tests.
   7. Recognize the need for preventive screening measures (e.g., cervical cancer screening).

Domain III. Analysis and Clinical Decision-making (34% of Exam)

1. Analyze assessment findings to determine their relevance to presenting complaints.[1]

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2. Recognize red flags (e.g., signs of stroke, heart attack, suicidal ideation, domestic abuse, trauma).
3. Establish and support the medical necessity for treatment.
4. Establish case management options (e.g., treatment, referral, co-management, discharge).
5. Differentiate between immediate, urgent, and routine referrals.
6. Recognize the potential impact of social determinants/drivers of health on treatment outcomes.
7. Recognize the potential impact of biomedical prognoses, management, and/or standard of care for common medical conditions on treatment outcomes.
8. Comprehend and utilize evidence-based information in clinical decision-making (e.g., research, expert opinion).

Domain IV. Professional Responsibilities (22% of Exam)

A. Professional and ethical conduct (5%)
   1. Comply with applicable federal laws and regulations (e.g., FDA, ADA, HIPAA, CDC).
   2. Comply with the legal scope of practice for Acupuncturists.
   3. Adhere to the *NCBAHM™ Code of Ethics*, *Grounds for Professional Discipline, and the Procedures for Upholding Professional Conduct*.
   4. Respect professional boundaries between patients and clinicians.
   5. Recognize and respond appropriately to unsafe or unprofessional practice.
   6. Treat all patients with empathy, dignity, and respect.
   7. Demonstrate respect for understanding non-discriminatory practices.
   8. Identify and manage potential conflict of interest situations.
   9. Recognize and work within the limits of own professional competence.
   10. Terminate the course of treatment as appropriate.

B. Infection control and workplace safety (3%)
   1. Identify modes of transmission of communicable diseases (e.g., airborne, fecal-oral).
   2. Follow CDC's standard and transmission-based precautions to prevent disease transmission and minimize the risk of infection.
   3. Maintain safe clinical environment in accordance with OSHA and other standards (e.g., cleaning and disinfection of equipment, hazard communication).

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4. Comply with workplace health and safety standards.

C. Records management and reporting (3%)
   1. Document the following information in patient's chart: assessment findings, treatment plan, responses to treatment and plan for follow-up care.
   2. Maintain patient records in accordance with professional and legal standards.
   3. Maintain patient's privacy and confidentiality of protected health information in accordance with HIPAA and other legislation.
   4. Report instances of suspected abuse and neglect as required by law.
   5. Report instances of communicable disease to public health department.

D. Insurance and billing (2%)
   1. Recognize the purpose and types of professional insurance (e.g., general liability, malpractice).
   2. Differentiate between ICD and CPT medical coding systems.
   3. Describe how basic ICD and CPT (e.g., E/M) codes are used in practice.

E. Communication and collaboration with patients, clinicians, and the public (4%)
   1. Communicate clearly and be responsive to the needs of patients, clinicians, and the public.
   2. Obtain informed consent for treatment from patients as required.
   3. Work collaboratively with other clinicians in the best interests of the patient.
   4. Communicate the commonalities and differences between acupuncture and biomedicine (e.g., correlation of diagnostic categories, differences in uses of organ systems).
   5. Explain acupuncture concepts to patients, clinicians, and the public using biomedical terminology.

F. Emergency management (5%)
   1. Recognize situations that require special care or emergency management (e.g., seizures, falls, anaphylactic reactions, hypoglycemia).
   2. Implement emergency protocols in practice (e.g., CPR).

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Appendix A. Medical Conditions

The examination will focus on but may not be exclusively limited to the conditions below.

Autoimmune

- Myasthenia gravis
- Psoriasis
- Rheumatoid arthritis (RA)
- Systemic lupus erythematosus (SLE)
- Temporal arteritis

Cardiovascular

- Aneurysm
- Angina pectoris
- Arrhythmia (e.g., atrial fibrillation, premature ventricular contraction, tachycardia)
- Atherosclerosis (e.g., coronary artery disease, peripheral vascular disease)
- Blood pressure disorders (e.g., hypertension, hypotension)
- Congestive heart failure
- Deep vein thrombosis
- Dyslipidemia
- Myocardial infarction
- Postural orthostatic tachycardia syndrome (POTS)
- Raynaud's phenomenon
- Valvular disease

Dermatological

- Burns
- Contagious skin conditions (e.g., lice, fungal infections, scabies, cellulitis)
- Noncontagious skin conditions (e.g., acne, eczema, alopecia, moles)

Endocrine and Metabolic

- Adrenal disorders (e.g., Cushing's, Addison's)

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- Diabetes Type 1
- Metabolic syndrome/insulin resistance (e.g., Diabetes Type 2)
- Obesity
- Parathyroid disorders
- Pituitary disorders
- Thyroid disorders (e.g., Hashimoto's thyroiditis, Graves' disease)

Gastrointestinal

- Appendicitis
- Cirrhosis
- Diverticular diseases (e.g., diverticulosis, diverticulitis)
- Food sensitivity/allergies (e.g., celiac disease, lactose intolerance)
- Gallbladder conditions (e.g., cholelithiasis, cholecystitis)
- Gastric bypass
- Gastritis
- Gastroesophageal reflux disease (GERD)
- Gastrointestinal polyps
- Hemorrhoids
- Hepatitis
- Hiatal hernia
- Inflammatory bowel disease (e.g., Crohn's disease, ulcerative colitis)
- Intestinal/biliary obstruction
- Irritable bowel syndrome (IBS)
- Pancreatitis
- Peptic ulcer (e.g., H. pylori, Campylobacter)
- Peritonitis
- Small intestinal bacterial overgrowth (SIBO)

Hematological

- Bleeding and coagulation disorders (e.g., pulmonary embolism)
- Disorders of platelets (e.g., thrombocytopenia)
- Disorders of red blood cells (e.g., anemia, polycythemia, sickle cell)
- Disorders of white blood cells (e.g., neutropenia)

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- Hemochromatosis

Infectious

- Bacterial infections (e.g., E. coli, strep, MRSA, impetigo)
- Foodborne illness
- Lyme disease
- Parasitic infections
- Sexually transmitted infections
- Tuberculosis
- Viral infections (e.g., shingles, measles, influenza, post-viral illness)

Genetics

- Down syndrome
- Ehlers-Danlos syndrome (EDS)
- Marfan syndrome

Mental, Neurodevelopmental, and Behavioral

- Addictions
- Anxiety disorders
- Attention-deficit/hyperactivity disorder (ADHD)
- Eating disorders (e.g., anorexia nervosa, bulimia nervosa)
- Mood disorders (e.g., depression, bi-polar disorder)
- Obsessive-compulsive disorder (OCD)
- Post-traumatic stress disorder (PTSD)
- Schizophrenia
- Spectrum disorder (e.g., autism)
- Substance use disorder (e.g., alcohol, opioids, nicotine)
- Suicidal ideation

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Musculoskeletal

- Acute, subacute, and chronic musculoskeletal conditions (e.g., trauma, degenerative disorders)
- Affecting the appendicular skeleton (e.g., meniscal injuries, frozen shoulder)
- Affecting the axial skeleton (e.g., disc herniation, TMJD)
- Bone density disorders
- Gout

Neurological

- Bell's palsy
- Concussion and traumatic brain injury (TBI)
- Dementia (e.g., Alzheimer's disease)
- Disorders of the central nervous system (e.g., cauda equina)
- Epilepsy
- Headaches (e.g., cluster, tension, migraine)
- Neuro-degenerative (e.g., multiple sclerosis, Parkinson's disease)
- Peripheral neuropathies
- Post-herpetic neuralgia
- Radiculopathies (e.g., sciatica)
- Stroke
- Transient ischemic attack (TIA)
- Trigeminal neuralgia
- Vertigo
- Other (e.g., Guillain-Barre, restless leg syndrome)

Oncological

- Cancer (e.g., organs, blood, bone, skin)
- Side effects secondary to cancer treatment (e.g., nausea, pain)

Ophthalmic and Ear, Nose and Throat

- Conditions of the ear (e.g., otitis media, tinnitus)
- Conditions of the eye (e.g., macular degeneration, conjunctivitis)

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- Conditions of the nose and throat (e.g., sinusitis, allergic rhinitis)

Pulmonary

- Asthma
- Chronic obstructive pulmonary disease (COPD)
- Pneumothorax
- Respiratory tract infections (e.g., bronchitis, pneumonia)

Reproductive

- Breast conditions (e.g., lumps, mastitis)
- Erectile dysfunction (ED)
- Infertility (e.g., polycystic ovarian syndrome (PCOS), low sperm count)
- Menopause (e.g., normal and with complications)
- Menstrual and uterine disorders (e.g., dysmenorrhea, endometriosis, fibroids)
- Perinatal support (e.g., labor preparation, postpartum)
- Pregnancy (e.g., normal and with complications)
- Prostate conditions (e.g., benign prostatic hyperplasia (BPH), prostatitis)

Urinary/Renal

- Dysuria (e.g., painful urination, retention)
- Incontinence
- Infections (e.g., cystitis, pyelonephritis)
- Kidney stones
- Pelvic floor dysfunction (e.g., pelvic floor prolapse)

Miscellaneous

- Implants (e.g., pacemakers, breasts, other body part enhancements, muscles that have been treated with Botox)
- Multi-system conditions (e.g., chronic fatigue, fibromyalgia)
- Sleep disorders (e.g., narcolepsy, sleep apnea, insomnia)
- Non-dermatological fungal infections (e.g., candidiasis)

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Appendix B. Pharmaceuticals

The examination will focus on but may not be exclusively limited to the list of medication categories below.

- Allergy/sinus
- Analgesic
- Anti-angina
- Anti-anxiety
- Antiasthmatic
- Antibiotic
- Anticancer
- Anticoagulant
- Antidementia
- Antidepressants
- Antidiabetic
- Antidiarrheal
- Antifungal
- Antihyperlipidemic
- Antihypertension
- Antinausea
- Anti-Parkinson
- Antipsychotics
- Antiseizure
- Antiviral
- Birth control
- Cannabinoids
- Central nervous system (CNS) stimulants/attention deficit
- Cough
- Dermatological
- Substances of abuse
- Fertility
- Gastrointestinal
- Hormonal replacement therapy
- Immune modulators/biologics
- Mood stabilizer
- Non-steroidal anti-inflammatory drugs (NSAIDs)
- Opioids
- Osteoporosis
- Sexual dysfunction
- Sleep
- Smoking cessation
- Steroids
- Stool softeners/laxatives
- Thyroid
- Weight management

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Appendix C. Supplements

The examination will focus on but may not be exclusively limited to the list below.

- Amino acids (e.g., L-glutamine, L-lysine, choline)
- Anabolic supplements (e.g., creatine)
- Antioxidants (e.g., coenzyme Q10, selenium)
- Bone/Joint health (e.g., glucosamine, collagen)
- Digestive support (e.g., enzymes, probiotics)
- Energy support (e.g., ashwagandha, guarana)
- Essential fatty acids (e.g., fish oils)
- Herbal supplements (e.g., saw palmetto, valerian, turmeric)
- Homeopathic remedies (e.g., arnica, nux vomica)
- Hormonal support (e.g., melatonin, wild yam, DHEA)
- Minerals (e.g., calcium, magnesium, potassium, iron)
- Mood support (e.g., St. John's Wort, SAMe, 5-HTP)
- Sexual function support (e.g., yohimbine)
- Vitamins (e.g., A, B1-B12, C, D, E, K)

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The Acupuncture with Point Location Content Outline

Effective September 1, 2026

Note to Candidate: This document serves as a guide to assist in examination preparation for candidates who have met NCBAHM™ eligibility requirements. Below is the content outline for the Acupuncture with Point Location examination.

Please note: In regard to Clean Needle Technique (CNT), the Acupuncture with Point Location module focuses on actual needling and its emergencies (e.g., needle angle and depth) in comparison to the Biomedicine module which focuses on universal precautions and emergency situations.

Domain I. Acupuncture Point Location (15% of Exam)

1. Identify points on images.
2. Identify points according to cun measurement and anatomical landmarks.

Domain II. Safety for Acupuncture Practice (15% of Exam)

1. Describe risks and benefits of acupuncture, moxibustion, and adjunctive therapies.
2. Recognize risk factors for individual patients (e.g., pregnancy, pacemaker, blood thinners, diabetes, hypoglycemia).
3. Recognize and manage adverse events (e.g., burns, pneumothorax, bruising, hemorrhage, needle shock, stuck needle).
4. Comply with Clean Needle Technique (CNT) standards and procedures.
5. Apply infection control procedures to acupuncture practice (e.g., CDC's Standard Precautions, OSHA standards).

Domain III. Treatment Planning (40% of Exam)

A. Selection of treatment methods (25%)
   1. Establish a treatment plan with appropriate treatment methods (e.g., acupuncture, cupping, moxibustion, combination of modalities).

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2. Adjust the treatment plan for specific patient populations (e.g., pediatrics, geriatrics, pregnancy, disabilities, history of trauma).
3. Evaluate patient's progress and modify the plan based on treatment response.
4. Discuss expectations for treatment with the patient (e.g., body sensations).

B. Acupuncture point selection (15%)
1. Identify actions and indications of acupuncture points.
2. Perform acupuncture point selection based on pattern differentiation, symptoms, and actions and indications of points.
3. Select points according to the following theories:
    a. *Channel (Jing Luo)*
    b. *Five Elements (Wu Xing)* (e.g., imbalances, excess-deficiencies, mother/son)
    c. *Organ (Zang Fu)*
    d. *Eight Principles (Ba Gang)*
    e. Point combinations (e.g., four gates)
    f. Tender points *(Ashi)*
    g. Myofascial trigger points
    h. Motor points
4. Select points according to the following point categories:
    a. *Five Transporting* (*Shu*)/*Antique* points (e.g., *Jing-Well, Ying-Spring, Shu-Stream, Jing-River, He-Sea*)
    b. Extra points[1]
    c. *Eight Influential* points
    d. *Eight Confluent* points
    e. *Four Seas* points
    f. *Front*-Mu (Alarm) points
    g. *Back-Shu* (Associated) points
    h. Source *(Yuan)* and Connecting *(Luo)* points
    i. Tonification and/or Sedation points
    j. *Five Elements* points (e.g., wood, fire, water)
    k. *Xi-Cleft* points
5. Select points according to the following acupuncture concepts:
    a. Auriculotherapy
    b. Local, adjacent, distal

[1] See Appendix A. Extra Points.

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c. Mirroring (e.g., elbow-for-knee, right-for-left, front-for-back)
d. Scalp acupuncture

Domain IV. Treatment Techniques (30% of Exam)

A. Acupuncture (15%)
   1. Identify indications, precautions and contraindications for acupuncture (e.g., pregnancy, organ damage, inflamed or non-intact skin).
   2. Apply appropriate patient positioning and draping procedures.
   3. Select needles to achieve desired treatment goals (e.g., length, gauge, filiform, or other types of needles).
   4. Apply appropriate needling techniques:
      a. Insertion (e.g., angle, depth, stretching skin)
      b. Manipulation (e.g., arrival of Qi, reinforcing, reducing, lifting, thrusting)
      c. Retention time
      d. Removal (e.g., order, speed, pressure)
   5. Apply myofascial trigger point (MTrP) techniques (e.g., local twitch response (LTR), dry needling, motor points).
   6. Adjust needling technique based on patient's response (e.g., stimulation method, intensity, needle retention time).

B. Moxibustion (5%)
   1. Identify indications, precautions and contraindications for moxibustion.
   2. Select the appropriate moxibustion technique (e.g., direct, indirect, warming needle).
   3. Adjust the moxibustion technique based on patient's response.

C. Adjunctive therapies (10%)
   1. Identify indications, precautions and contraindications for adjunctive therapies.
   2. Select the appropriate adjunctive technique:
      a. Bloodletting
      b. Cupping
      c. Ear seeds, balls, magnets
      d. Acupuncture with electrical stimulation
      e. Guasha
      f. Tuina
      g. Acupressure

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h. Heat application (e.g., heat lamp)

3. Modify the adjunctive technique based on patient's response.

Appendix A. Extra Points

The examination will focus on but may not be exclusively limited to the list below.

Anmian
Bafeng
Baichongwo
Bailao
Baxie
Bitong
Dannangxue
Dingchuan
Erbai
Erjian
Heding
Huatuojiaji
Jianqian/Jianneiling
Lanweixue
Luozhen
Pigen
Shiqizhuixue/Shiqizhuixia
Shixuan
Sifeng
Sishencong
Taiyang
Tituo
Xiyan/Neixiyan
Yaotongxue
Yaoyan
Yintang
Yuyao
Zigong

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Suggested Bibliographies

The Content Outlines available on the NCBAHM website are the primary resources for studying for the examinations. The purpose of the following suggested bibliographies is to provide candidates with resources to assist in preparing for the NCBAHM examinations.

There is no single text recommended by the NCBAHM. All NCBAHM examinations reflect practice in the United States as determined by the most recent job analysis. Candidates should feel free to consider other resources that cover the material in the Content Outlines.

The NCBAHM™'s Examination Development Committees (EDCs) frequently use the following texts as resources for writing and referencing items; however, the sources used are not limited to the books listed here. The NCBAHM does not endorse any third-party study or test preparation guides.

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Websites

Federal regulations will take precedence over information found in other references.

Centers for Disease Control and Prevention (CDC)
https://www.cdc.gov/
- Infectious Disease
- Infection Control
- First Aid
- NIOSH

Centers for Medicare & Medicaid Services (CMS)
https://www.cms.gov/
- Charting and Coding

Medline Plus
https://medlineplus.gov/
- Drugs, Herbs and Supplements

National Center for Complementary and Integrative Health (NCCIH)
https://nccih.nih.gov/
- Herbs at a Glance
- Health Topics A-Z

NCBAHM™ Code of Ethics

NCBAHM™ Grounds for Professional Discipline *(located under Professional Discipline section)*

NCBAHM™ Procedure for Upholding Professional Conduct *(located under Professional Discipline section)*

Occupational Safety and Health Administration (OSHA)
https://www.osha.gov/
- Universal Precautions
- Office Policies and Procedures

Office of Disease Prevention and Health Promotion
https://odphp.health.gov/
- Healthy People 2030
  - Social Determinants of Health

U.S. Department of Health & Human Services (HHS)
https://www.hhs.gov/
- Medical Records
- Laws and Regulations
  - HIPAA
- National Institutes of Health, Office of Dietary Supplements
  https://ods.od.nih.gov/factsheets/list-all/
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World Health Organization (WHO)
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Examination Nomenclature Cross-Reference

Historically, the translation of Traditional Chinese Medicine (TCM) terminology into the English language has led to the use of different English terms for some TCM concepts. The following list is intended to provide the most commonly used terms from the most often studied references, as cited below. **It is not intended to be all-inclusive.** It is also provided in the form of a glossary in the English language version of Foundations of Oriental Medicine, Acupuncture, and Chinese Herbology modules/examinations.

1. Theory

Qi: Vital Energy / *Chi* / *Ki*
Wu Xing: Five Elements / Five Phases
Sheng Cycle: Generating / Promoting / Interpromoting / Engendering / Mutual Production
Ke Cycle: Controlling / Acting / Interacting / Restraining / Mutual Checking / Mutual Control / *Ko*
Cheng Cycle: Overacting / excessive Acting
Wu Cycle: Counteracting / Insulting

2. Physiology

Yuan Qi: Original / Primary Qi
Zong Qi: Pectoral / Gathering / Ancestral Qi / Qi of the Chest
Zhen Qi: True / Normal / Original / Vital Qi
Ying Qi: Nutrient / Nutritive / Construction Qi
Wei Qi: Defensive / Protective / Defense Qi
Zhong Qi: Middle / Central Qi
Zheng Qi: Antipathogenic / Righteous / Upright Qi
Xie Qi: Pathogenic / Evil Qi
Qing Qi: Clear Qi
Da Qi: Great Qi / Qi of the environment / Air
Gu Qi: Food / Grain Qi
Jing: Essence
Xue: Blood
Jin Ye: Body Fluids / Fluids / *Jing Ye*
    Jin: liquid / thinner fluids (clear, light, thin-watery)
    Ye: humor / thicker fluids (turbid, heavy, dense)
Zang Fu: organs
    Zang: viscera / Yin organs

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Fu: bowels / Yang organs
San Jiao: Triple Burner / Triple Warmer / Triple Energizer
Shen: Spirit / Mind

3. Pathology and Diagnosis

Xu: deficient / deficiency / vacuous / vacuity / empty
Shi: excess / repletion / full
Zhong Qi Xia Xian: Collapse of Central Qi / Qi Sinking / Center Qi Fall
Shui Zhong: Water Swelling / Water Qi / Edema
Tan Yin: Damp Phlegm / Phlegm Rheum / Phlegm-Fluid
Nei Yin: internal causes, endogenous / internal pathogenic factors
Wai Yin: external causes, exogenous / external pathogenic factors
Bu Nei Bu Wei Yin: neither internal or external causes / miscellaneous factors

She Tai: tongue coating / tongue moss / tongue fur
    Hua Tai: glossy / watery
    Jing Mian She: peeled / mirror / mirror-like, no coating
    Ni Tai: greasy / sticky / slimy
She Ti: tongue body / tongue proper
    She Pang Da: swollen / flabby / enlarged
    Shi Bian Chi Hen: tooth marks / teeth marks / scalloped / dental impressions on the margins of the tongue
Pulse locations:
    Cun: Distal / Inch / *Tsun*
    Guan: Middle / Gate / Bar / *Kuan*
    Chi: Proximal / Cubit / Foot / *Chih*
Bing Mai: abnormal pulses / 28 pulse qualities
    Fu Mai: hidden
    Chen Mai: deep / sunken / sinking
    Chi Mai: slow
    Shuo Mai: rapid / *Shu Mai*
    Hong Mai: flooding / tidal / surging
    Wei Mai: minute / faint / indistinct
    Xi Mai: thin / thready / fine (synonymous with *Xiao Mai* / small pulse)
    San Mai: scattered / dissipated
    Xu Mai: weak / empty / vacuous / deficiency
    Shi Mai: full / excess / replete
    Hua Mai: slippery / rolling
    Se Mai: choppy / rough / hesitant
    Chang Mai: long
    Duan Mai: short

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Xian Mai: wiry / string-like / bowstring / stringy / string-taut / taut / *Xuan Mai*
Kou Mai: hollow / scallion-stalk (onion, leek)
Jin Mai: tight / tense
Huan Mai: moderate / leisurely
Ge Mai: leather / drum-skin / tympanic
Lao Mai: confined / firm / sunken-full / fixed / prison
Ruo Mai: weak / feeble / frail
Ru Mai: soggy / soft
Fu Mai: floating / superficial
Dong Mai: moving / spinning bean / stirred
Ji Mai: swift / racing
Cu Mai: hurried / abrupt / skipping / rapid / irregular
Jie Mai: knotted / bound / slow and irregular
Dai Mai: intermittent / regularly intermittent / regularly Interrupted / regularly irregular

4. Technique

Bu: supplement / tonify / boost
Xie: reduce / drain / sedate / disperse

1. Channels and Points

Jing Luo: channels & network vessels
Jing Mai: channel / meridian
Shi Er Jing Mai: 12 primary channels / regular channels / meridians
Luo Mai: connecting channels / collaterals / network vessels,
Bie Luo: 15 divergent network vessels / channel divergences
Jin Mai: tendinomuscular meridians / muscle or sinew channels
Ba Mai: 8 Extra Meridians / 8 Vessels / Extraordinary Vessels / Ancestral Vessels
Ren Mai: Conception Vessel / CV / *Directing / Ren Mo*
Du Mai: Governing Vessel / GV / Governor / *Du Mo*
Chong Mai: Penetrating Vessel / Thoroughfare
Dai Mai: Girdle Vessel / Belt / Girdling /*Tai Mo*
Qiao: Heel Vessel / Stepping / Motility / *Chiao*
Wei: Linking Vessel
Ba Mai Jiao Hui Xue: confluent points / confluence points / master points of the 8 vessels / opening points of the 8 extra meridians
Jiao Hui Xue: intersection points
Cun: inch / body inch / A. C. I. (anatomical Chinese inch) / *Tsun*
Luo point: connecting / junction point / *Lo*
Yuan point: source / primary point
Xi point: cleft / accumulation point

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Wu Shu points = 5 transport / transporting / antique / command points
- *Jing*: well
- *Ying*: spring / brook / gushing
- *Shu*: stream / transporting
- *Jing*: river / traversing
- *He*: sea / uniting

Shu points: Back-Shu / Back transporting / A.E.P. (Associated Effect Point) / *Yu*
Mu points: Front-Mu / front collecting / alarm point
Mother point: reinforcing / tonification point
Child point: son / reducing / sedation / dispersing / draining point
Jiao Hui Xue: coalescent / crossing / intersection points
Ba Hui Xue: 8 meeting points / 8 influential points
Zi Wu Liu Zhu: horary cycle / Chinese clock /organ clock / midday-midnight cycle / stem and branches point selection

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Sample Questions

The following sample questions represent different types and levels of items that may appear on the NCBAHM examinations. These questions do not necessarily represent the difficulty level of the items candidates will receive on the examination, nor do they reflect the percentage of items candidates will receive in each content area. These sample questions merely reflect the possible format and variety of items, which may assist candidates in preparing for the NCBAHM examinations. Performance on the sample questions is not an indicator of performance on the NCBAHM examinations.

The answers to the sample questions appear at the bottom of the page containing the sample questions for the Acupuncture with Point Location section.

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Foundations of Oriental Medicine

FOM-1
According to Five Element theory, which taste, color, and organ are associated with Metal?

(A) bitter, red, Lung
(B) pungent, white, Lung
(C) spicy, yellow, Spleen
(D) sweet, yellow, Spleen

FOM-2
A 29-year-old patient complains of hypochondriac pain and fullness for several months. She is also experiencing dry mouth and throat, depression, moodiness, scanty menstrual flow, and breast pain. She has a pale tongue and a thready, wiry pulse. Which of the following is the most appropriate diagnosis?

(A) Liver Fire insulting Lung
(B) Liver Qi stagnation transforming to Fire
(C) Liver Qi stagnation with Blood deficiency
(D) Liver Fire attacking Stomach

FOM-3
A patient complains of shortened menstruation with scanty, dull red, clear, thin menses. She has coldness in the lower abdomen. Her tongue is pale, tender, with white fur. Her pulse is deep and tight. Which of the following is the most appropriate treatment principle?

(A) activate the channel and clear Heat
(B) tonify Yang and move Blood
(C) tonify Yin and clear Heat
(D) warm the channel and expel Cold

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Biomedicine

BIO-1
A 40-year-old patient with an enlarged thyroid gland is most likely deficient in which of the following?

(A) iodine
(B) iron
(C) magnesium
(D) zinc

BIO-2
A parent reports that their active eight-year-old child has been fussy, thirsty, and tired for the past 24 hours. The parent also states that the child complains of a headache and constipation. The child's blood pressure is low with a rapid pulse. Which of the following is most likely to be suspected?

(A) anxiety attack
(B) dehydration
(C) food poisoning
(D) hyperthyroidism

BIO-3
A lethargic, 53-year-old patient fell and hit their head six hours before their appointment. The patient now presents with confusion, difficulty remembering the event, and has vomited twice since the fall. Which of the following is the best course of action for this patient at this time?

(A) treat the patient and recommend that they consult their physician
(B) treat the patient and retain them in the office for observation
(C) do not treat the patient, but refer them to a neurologist within 72 hours
(D) do not treat the patient, but refer them immediately to the emergency department

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Acupuncture with Point Location

ACPL-1
Which of the following points could be needled with the patient positioned in the prone position?

(A) Yintang (Extra)
(B) P 2 (Tianquan)
(C) Sp 11 (Jimen)
(D) GB 36 (Waiqiu)

ACPL-2
For which of the following conditions is the bleeding technique most likely indicated?

(A) high fever
(B) chronic asthma
(C) anemia
(D) diabetes

ACPL-3
Which of the following statements best describes the location of Lu 7 (Lieque)?

(A) on the forearm, superior to the styloid process of the radius, 1 cun proximal to the transverse crease of the wrist
(B) on the forearm, superior to the styloid process of the radius, 1.5 cun proximal to the transverse crease of the wrist
(C) on the radial side of the flexor carpi ulnaris tendon, 1 cun proximal to the transverse crease of the wrist
(D) on the radial side of the flexor carpi ulnaris tendon, 1.5 cun proximal to the transverse crease of the wrist

Answers:

FOM-1 = B	BIO-1 = A	ACPL-1 = D
FOM-2 = C	BIO-2 = B	ACPL-2 = A
FOM-3 = D	BIO-3 = D	ACPL-3 = B

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Frequently Asked Questions

Examination Administration Frequently Asked Questions and Answers (Q&A)

The National Certification Board for Acupuncture and Herbal Medicine (NCBAHM), formally known as National Certification Commission for Acupuncture and Oriental Medicine (NCCAOM), a national, nonprofit certification organization, has compiled the following questions and answers to assist candidates and school representatives in understanding its examination administration policies and procedures. The NCBAHM is committed to maintaining the integrity and fairness of the NCBAHM examinations, in order that they serve as meaningful measures of entry-level competence to practice acupuncture and herbal medicine. NCBAHM's commitment to public safety cannot be overstated; it is a requirement of our mission, which is *to assure the safety and well-being of the public and to advance and advocate for the professional practice of NCBAHM Board-Certified Acupuncturists™ by promoting established national standards focused on competence and credentialing.*

About the Examinations

1. What is the format of the examinations?

The NCBAHM examinations are administered in English and delivered using a computer-adaptive format. This format enables continuous year-round scheduling and testing, and the generation of preliminary results on-site immediately following completion. Test takers are provided 2.5 hours to answer 100 multiple-choice items. Starting September 1, 2026, the updated content outlines, based on the 2024 national Job Analysis study, will take effect.

Computer adaptive testing (CAT) has been widely used across healthcare and professional certification organizations and is recognized as an efficient and reliable testing method. In a computer adaptive examination, questions are administered according to the candidate's ability. This means that a candidate's response to a question determines the next question they receive. If a question is answered correctly, the next question presented is more difficult. If a question is answered incorrectly, the next question presented is easier. Because

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the correct or incorrect response to each question is used to select the next question, candidates are unable to change their responses to previous questions.

CAT offers several benefits such as access to year-round testing, no examination registration deadlines, preliminary examination results provided at the test center, expedited examination results delivery to state licensing Boards (upon candidate request) and more precise measurement of candidate ability.

2. How do I prepare for an examination?

The NCBAHM provides a comprehensive *NCBAHM™ Candidate Preparation Handbook* (NCBHAM Website) for each certification program. Each Handbook includes an overview of the examination process, the examination content outlines, test specifications, suggested bibliographies, a nomenclature list (e.g., a cross-reference of terms that are frequently used), and sample examination questions. The Handbooks are available for download from the Exam Preparation Center section of the NCBAHM website. The updated content outlines, based on the 2024 national Job Analysis study, will go into effect on September 1, 2026.

The NCBAHM also offers the official NCBAHM Practice Tests™ to assist candidates in preparing for the NCBAHM Certification Examinations. For more information, please visit Official NCBAHM Practice Tests™ (NCBAHM Website).

3. For the Acupuncture with Point Location examination, are the acupuncture point number and pinyin provided?

Yes. The acupuncture point number and pinyin are provided for the points [e.g., CV (Ren) 6 (Qihai), Erbai (Extra), etc.].

4. Does the NCBAHM publish a list of single herbs and herbal formulas that may be tested on the Chinese Herbology examination?

Yes. Appendices of single herbs and herbal formulas are included within the Chinese Herbology (CH) content outline. Botanical name (in English) and pinyin will be provided with the herbs and herbal formula names [e.g., Chai Hu (Bupleuri Radix), Cang Er Zi San (Xanthium Powder)] as indicated in the appendices. Content outlines can be accessed from the Exam Preparation Center section of the NCBAHM website.

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Scheduling an Examination

Approved Candidate:

An Approved Candidate is one who has met the NCBAHM eligibility requirements and has been authorized to register for the NCBAHM examinations. Approved Candidates will receive notification via email notifying them that their Authorization to Test (ATT) letter is available in the candidate's online account on the NCBAHM Certification Portal. It is imperative that the candidate keep their physical address, email address, and name changes updated and current with the NCBAHM. The ATT letter is especially important because it contains the information needed to schedule examinations and gain access to the Pearson VUE testing center.

Note: Any questions regarding a candidate's ATT letter or whether a candidate is an Approved Candidate, please email info@ncbahm.org.

5. When and how can I schedule an examination?

It is highly recommended that NCBAHM examinations should ONLY be scheduled when candidates are ready to take the test.

Candidates may schedule their NCBAHM examination at any time after approval. Once a candidate receives an Authorization to Test (ATT) letter through their NCBAHM Certification Portal account, they can register and select any available time at their preferred Pearson VUE test center. All required examinations must be completed and passed within four (4) years of the application submission date.

The candidate's ATT letter includes registration information and detailed instructions on how to schedule an examination. NCBAHM examinations are administered at over 250 Pearson VUE Professional Test Centers worldwide. Candidates can schedule an NCBAHM examination by calling Pearson VUE directly at (888) 235-7649, Monday through Friday, 7am-7pm Central Time (CT), or by scheduling online at Pearson VUE NCBAHM Certification Testing. Payment is made directly with Pearson VUE using accepted credit cards at the time of scheduling examinations.

6. Can I reschedule an examination?

Candidates must cancel or reschedule their examination at least one full business day (24 hours) in advance of the scheduled examination appointment by contacting Pearson VUE at 888-235-7649, Monday through Friday, 7 a.m. - 7 p.m. Central Time (CT), or online at www.pearsonvue.com/ncbahm. During the cancelation/rescheduling process, Pearson VUE

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will provide a confirmation number. It is imperative this confirmation number be maintained by the candidate in case of a cancelation/rescheduling dispute. In addition, the candidate will receive a confirmation email from Pearson VUE within one full business day of scheduling/rescheduling/canceling an examination. If this email confirmation is not received, the transaction is incomplete. Failure to cancel or reschedule within the required timeframe or failure to appear for the scheduled examination appointment will result in forfeiture of all examination fees paid.

Please note that with the initial NCBAHM Certification application, candidates receive three (3) free cancelation or reschedule examination attempts for each examination. After the third (free) cancelation or reschedule, the candidate must appear on the rescheduled examination date and time; otherwise, the examination appointment will go to "No Show", and the candidate will forfeit examination fees paid. Additional cancelation or rescheduling attempts can be obtained for a nominal fee.

Taking the Examination

7. Is it true that the computer screen turns off after 5 minutes, from the time the computer is started by the test center proctor, if the first question is not answered?

Yes. This action is also declared in the *NCBAHM™ Certification Handbook* that candidates agreed to read. The first question on the computer screen is a legal agreement that states the candidate will treat everything seen on the examination with the utmost and absolute confidentiality. **The candidate will have 5 minutes to review the Non-Disclosure Agreement (NDA) and select *"I agree"* to begin the examination**. To ensure that candidates are prepared for this step, please read the full text of the *Non-Disclosure Agreement and Full Terms of Use for the NCBAHM Examination* below.

If the candidate does not accept this NDA presented on-screen within the allotted 5 minutes, the examination will terminate, and the candidate will be locked out of the computer. In addition, the candidate will not be allowed to continue with the examination and **fees for that examination will not be refunded**. According to Pearson VUE rules, candidates may not begin writing on the erasable board until the test has begun. **Therefore, candidates should not use their erasable boards until after they have agreed to the NDA.** For example, if the examination terminates because the candidate was writing on their erasable board during the NDA, Pearson VUE is unable to restart the examination. The candidate will forfeit all the fees paid for the examination, and the candidate will have

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to re-register and pay full fees for the examination if they wish to take it again. For more information, please contact info@ncbahm.org.

Non-Disclosure Agreement (NDA) and General Terms of Use for NCBAHM *Examinations*

"I have read and understand the Examination Instructions. I have agreed to abide by the NCBAHM™ *Code of Ethics and Grounds for Professional Discipline and acknowledge that if I am caught cheating on this examination, including the sharing of information after the examination is complete; I will be subject to review by the Professional Ethics and Disciplinary Committee of* NCBAHM. *If I am found to have violated the Code of Ethics and Grounds for Professional Discipline, I understand that my scores will be canceled, and I may not have the opportunity to test again.*

Additionally, I understand that this examination is confidential and is protected by trade secret law. It is made available solely for the purpose of becoming certified by NCBAHM. *I am expressly prohibited from disclosing, publishing, reproducing, or transmitting this examination, in whole or in part, in any form or by any means, verbal or written, electronic or mechanical, for any purpose.*

I am the candidate whose name appears on the initial screen and as an affirmation to the Statement of Acknowledgement I signed when submitting my application. I acknowledge that I am prohibited from transmitting information about NCBAHM *examination questions or content in any form to any person or entity. I also acknowledge that if I suspect a violation on the part of others, it is my responsibility to report these actions to the* NCBAHM.*"*

The NCBAHM is committed to the integrity and security of its examinations. Candidates have a duty to maintain strict confidentiality with respect to the content of the examinations and comply with all examination security policies and procedures. Any breach of confidentiality that may compromise the security of the examination content (e.g., sharing or receiving information about the examination from another person including teachers/professors, before, during, or after the examination) will be grounds for disciplinary action, including but not limited to denial or revocation of certification by the NCBAHM.

8. Do all the questions have to be answered on an examination?

Candidates must answer all the questions for the examination to be scored. Because the examination utilizes computer adaptive testing (CAT), candidates are unable to skip questions or return to previous questions once they move forward onto the next question. See the answer to Question #1 for additional information.

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9. What happens if there are computer technical difficulties or a power outage at the test center?

On rare occasions, technical difficulties at the test center may be encountered. Candidates should immediately notify the test center proctor if any computer irregularities arise. If the test center experiences an unexpected, temporary power outage during examination administration, test center back-up systems are in place to ensure testing data is preserved. The candidate's examination will resume from the point where it was interrupted, and the candidate continues with the examination. Every effort is made to administer the examination as scheduled. However, should an examination be canceled at a testing center, all scheduled candidates will be notified by e-mail or phone with rescheduling instructions.

10. I have a concern with one of the questions on the examination, what is the procedure to have this question reviewed?

An unsuccessful examination result alone is not sufficient grounds to submit a request. At the test center, candidates can file an incident report regarding examination content concerns with the test center proctor while the item is still fresh in their mind. The test center proctor is only allowed to record the question (item) number as they are not allowed to record any examination content specifics or description of the examination items. Be sure to obtain a copy of the incident report number. Candidates may submit questions or comments about specific aspects of the examination content in writing to NCBAHM (examcontent@ncbahm.org) within 30 days of taking the examination as indicated in the Examination Content Review Process section of the NCBAHM website and in the *NCBAHM™ Certification Handbook*. Only individual items will be reviewed, and reviews are not conducted on a candidate's entire examination. The written inquiry must include the candidate's name, NCBAHM ID, email address, test date and location, examination taken (FOM, ACPL, BIO, CH) and a description of the specific item (e.g., an item involving a pregnant patient could have more than one indicated correct answer or no correct answer is indicated).

The request will first be reviewed by the NCBAHM Testing Department to determine if the submitted question or comment is to be presented to the NCBAHM™ Examination Review Committee for a final determination. Please note that submission of a request does not automatically lead to a rescoring of the examination or Committee review. If the question under review is found to have a discrepancy, the candidate's examination will be rescored.

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The candidate will be notified in writing within 45 business days of any resulting action of the inquiry. All determinations of the NCBAHM™ Examination Review Committee are final.

Please note that no information regarding the specific question will be discussed with candidates, including whether the candidate's response was correct or incorrect. The NCBAHM never releases copies of examinations or individual examination questions. This is considered best practice within the licensure/certification testing industry and the NCBAHM must protect the integrity of its examination – most especially its content. As the NCBAHM's mission is to ensure the safety and wellbeing of the public, we take the security of our examination content very seriously and therefore never share any specific information about questions with candidates.

It is imperative that the candidate refrain from discussing the content of the examination question with anyone other than the NCBAHM **Testing Department. Doing so is a violation of the** NCBAHM™ ***Non-Disclosure Agreement (NDA) and the NCBAHM™ Code of Ethics*****.** The NCBAHM is committed to the integrity and security of its examinations. Candidates have a duty to maintain strict confidentiality with respect to the content of the examinations and comply with all examination security policies and procedures. Any breach of confidentiality that may compromise the security of the examination content (e.g., sharing or receiving information about the examination from another person including teachers/professors, before, during, or after the examination) will be grounds for disciplinary action, including but not limited to denial or revocation of certification by the NCBAHM.

11. There were questions on my examination that were not taught at my school, can you explain?

Please keep in mind that the NCBAHM offers **national** certification examinations. NCBAHM conducts a Job Analysis (JA) every five to seven years to validate and ensure that its NCBAHM entry-level certification examinations are reflective of current practice in all regions of the United States and not just one geographical region. NCBAHM examinations are derived from the results of the JA survey, which creates the content outlines of each examination module. NCBAHM examinations do not match school curriculum as that is not the job of a national certification examination. It measures entry-level competency of current practice and not what schools are teaching. It would be difficult to match the curriculum of 44 schools. Furthermore, a national certification examination is not synonymous with a specific academic program's comprehensive examination content. For more information, please read an article from *Acupuncture Today*.

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Examination Attempts

12. I was unsuccessful in passing the examination, when can I retake the examination?

Candidates who are unsuccessful in passing an examination 1 - 4 times must wait 45 calendar days after their last attempt before retaking the same examination module. The NCBAHM recommends that candidates give themself sufficient time to prepare before retesting. For updated information on examination attempt limits, refer to "Re-taking an Examination" on the *When Taking Board Exams* page from the NCBAHM website.

13. Is there a limit to the number of times an examination can be taken?

Candidates have five (5) opportunities to pass an examination once their graduate transcript and Clean Needle Technique (CNT) Certificate of Completion are received at NCBAHM. After the fifth unsuccessful attempt to pass a NCBAHM examination (inclusive of all formats), the candidate must meet additional educational requirements based on multiple factors (i.e., certification application validity, etc.) to receive additional examination attempts. For additional information refer to "Re-taking an Examination" on the *When Taking Board Exams* page on the NCBAHM website or email examattempts@ncbahm.org.

Examination Scoring and Results

14. What is a scaled score?

A candidate's raw score—the number of questions answered correctly—is converted into a scaled score that adjusts for question difficulty. Because adaptive examinations present different questions to each test-taker, scaled scoring ensures fair comparisons across candidates and examination versions. Scaled scores range from 1 to 99, with 70 as the passing standard. They should not be interpreted as the percentage of questions answered correctly. For additional information, read *Standard Setting, Scaling and Equating document (PDF)*, accessible on the "Examination Results" section from the When Taking Board Exams page on the NCBAHM website.

15. When will I receive my examination results?

Upon completion of an adaptive format examination at a Pearson VUE test center, candidates receive **preliminary** result on-screen regarding whether they were successful or unsuccessful in passing the examination. These examination results are considered **preliminary** and are verified by an independent third-party testing company before **official** notification is made. The official notification is uploaded to the candidate's online account

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on the NCBAHM Certification Portal within 7 - 10 days after the examination date, which is available for download. Candidates can track their examination results within their online account on the NCBAHM Certification Portal.

16. What areas do I need to focus on for the next examination when I was unsuccessful in passing?

Candidates should use the scaled scores in the Examination Content Area Diagnostic Report, included in their official examination result letter to identify areas needing improvement. It is recommended that candidates review the NCBAHM™ Content Outlines (NCBAHM Website) in preparing for their next examination. The NCBAHM Testing staff are not subject matter experts and cannot provide any guidance on how to study for the examination.

The NCBAHM strongly encourages any candidate who was unsuccessful in passing to seek guidance on how to prepare for the examination. Speak with a school program director, a faculty member, or mentor. The NCBAHM™ *Candidate Preparation Handbook,* which contain the Content Outlines and a suggested bibliography of study references, are valuable resources and can be downloaded from the NCBAHM website under the Exam Preparation Center section.

17. The average of my content area scaled scores was 70 (or higher), why did I receive an unsuccessful status?

The overall **scaled score** cannot be calculated from the content area scaled scores provided on the candidate's Examination Content Area Diagnostic Report. Because each content area listed on a content outline contains varying numbers of questions, averaging those scores is different from the reported overall scaled score. The overall scaled score is based solely on the candidate's performance on the entire examination.

For additional information, read *Standard Setting, Scaling and Equating document (PDF)*, accessible on the "Examination Results" section from the Examination Process page on the NCBAHM website.

18. How do I request a score verification of my examination results?

Candidates may request that their examination score be verified within 30 days of receiving initial notification that a candidate was unsuccessful in passing the examination. The score verification process is a manual check of the computer's scoring, conducted by NCBAHM Testing staff, to ensure that the candidate's responses were accurately recorded. **Score**

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verification does not include a review of examination content or reconsideration of the correct answer to any item. Individual items and examination content will not be discussed or considered during the score verification process. To request a score verification, the candidate must complete and submit the NCBAHM™ Score Verification Request form (NCBAHM Website). An invoice will be issued to the candidate's online account on the NCBAHM Certification Portal for payment. There is a nominal fee per examination for score verification.

Please note that the NCBAHM does not encourage score verification requests. The NCBAHM enforces strict quality control procedures to ensure examination results are accurate before they are released to candidates. Two professional testing companies score every examination independently to ensure each item is scored accurately and the overall score is correct. In addition, both testing companies have multiple checks and balances in place to flag anomalies in test data that require investigation. Due to the thoroughness of the NCBAHM's examination scoring procedures, no errors have ever been identified from a score verification request.

19. I passed the examination. What was my score?

Candidates who pass the examination do not receive a total score or a breakdown of their performance in each content area. NCBAHM examinations are designed to assess entry-level competence to practice, not to measure excellence.

The purpose of the Examination Content Area Diagnostic Report is to assist candidates who were unsuccessful in passing the examination, understand their strengths and areas of improvement and help focus study efforts.

20. How many candidates passed the examinations on their first attempt?

Please refer to the chart below for 2020-2025 examination pass rates for first-time takers from Certification Route 1: Formal Education: United States Applicants.

Examination Module	2020*	2021	2022	2023	2024	2025
Acupuncture with Point Location	74.2	70.5	73.3	71.0	75.4	80.2
Biomedicine	72.3	73.2	69.0	64.5	70.1	68.7
Chinese Herbology	63.0	69.5	68.9	65.6	66.2	62.3
Foundations of Oriental Medicine	95.4	96.1	93.3	93.3	93.4	92.2

*Implementation of new content outlines in 2020

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Obtaining Certification

21. I passed all the required examinations. Does this mean I am now certified?

Passing all required NCBAHM examinations does not automatically result in certification. Candidates are certified and become a Diplomate of NCBAHM after all required documents are accepted. Then, their name appears on the NCBAHM™ Registry (NCBAHM Website). See the NCBAHM™ *Certification Handbook (*NCBAHM *Website)* for information about the documents required to become certified.

Diplomates may represent or advertise their NCBAHM certification to employers, insurance companies and the public by directing them to the NCBAHM™ Registry (NCBAHM Website). Certified Active Diplomates will receive the official NCBAHM congratulatory letter, the wallet-size identification card, and a wall certificate suitable for framing approximately six (6) weeks after certification (active Diplomate status) is achieved. USPS maintains tracking information for only 90 days. Candidates who have not received their certificate and ID card at ten (10) weeks after certification, please email the NCBAHM at info@ncbahm.org

It is the candidate's responsibility to submit a request to the NCBAHM via their online account on the NCBAHM Certification Portal for their results to be sent to any regulatory agencies.

22. How long are my examination results valid?

Continued Active Diplomate status through initial certification and continued recertification ensures that examination results do not expire.

A limit is placed on the number of years an examination result is valid for the purpose of NCBAHM certification. The NCBAHM grants up to 12 years for first time candidate examination validity.

Status:	Examination Score Validity for Initial Certification:
Certified Diplomate - Active	No expiration date
Former Certified Diplomate - Inactive	No expiration date
Former Diplomate - Lapsed	3 years from active certification
Former Diplomate - Terminated	All examination results expired
Candidate - Application in Process	12 years from examination date

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Candidate - Reapply Application in Process	12 years from examination date
Candidate - Application Expired	12 years from examination date

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